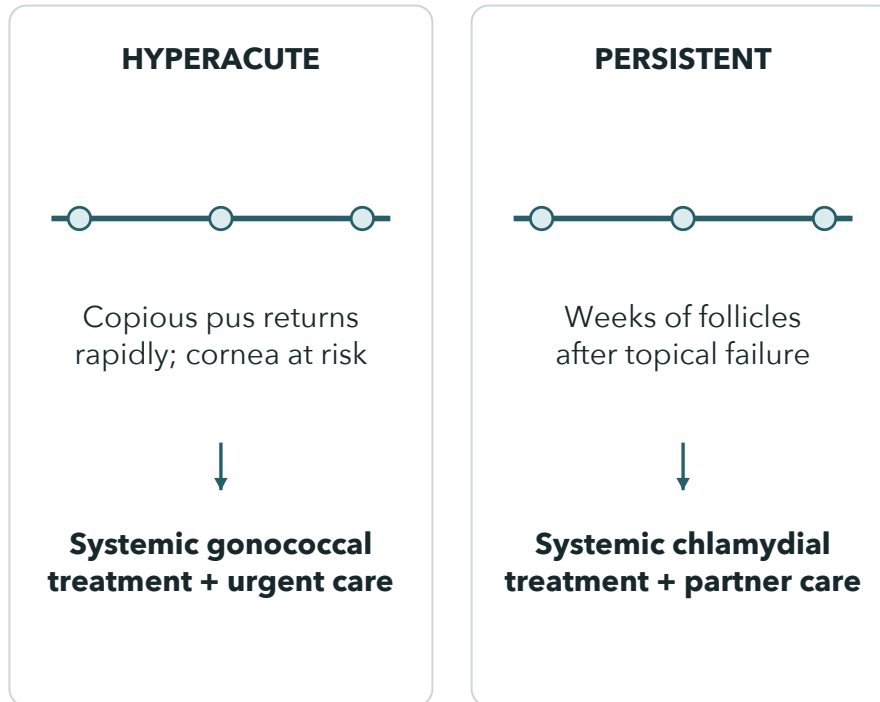


Conjunctivitis that needs escalation

Hyperacute infection, chronic inclusion disease and corneal allergy complications.

A topical-only plan can miss the source

A | Compare the time course



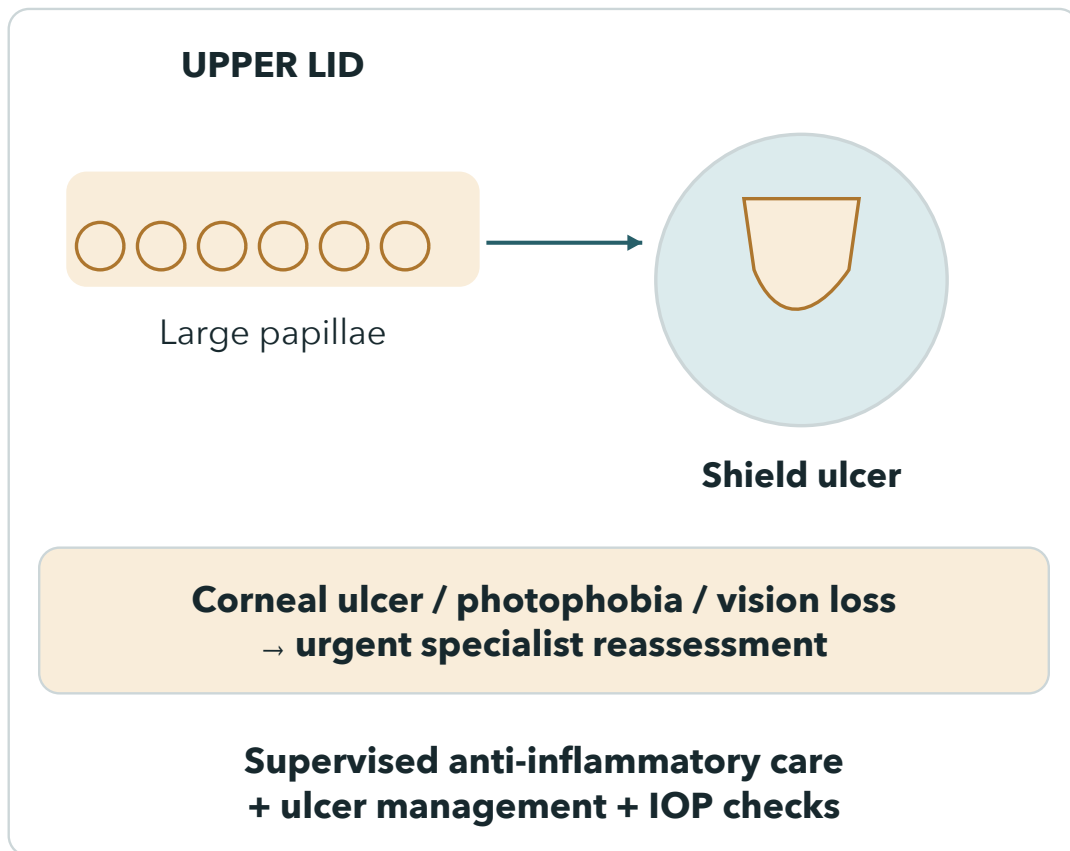
Original comparison timeline. Clinical severity and exposure determine the pathway while specimens guide organism-specific care.

Systemic treatment cues

Pattern	Action
Copious pus reforms rapidly	Consider gonococcus; urgent ocular assessment, cultures or NAAT and prompt systemic ceftriaxone.
Weeks of follicles; topical failure	Consider adult inclusion disease; appropriate testing and systemic anti-chlamydial therapy.
No genital symptoms	Does not exclude associated infection. Coordinate sexual-health care and partner assessment.
Ulcer or reduced vision	Corneal involvement changes urgency; routine conjunctivitis advice alone is insufficient.

Allergy becomes a corneal problem

B | Papillae and shield ulcer



Original lid and corneal surface schematic. A shield ulcer is a complication requiring corneal assessment, not proof of bacterial infection.

Use supervised anti-inflammatory care

Dual-action antihistamine and mast-cell stabilising drops suit uncomplicated allergic disease, but a shield ulcer needs specialist review. Treatment may include a monitored steroid course, steroid-sparing therapy and removal of an adherent plaque when indicated. Exclude infection and check pressure during steroid treatment. Avoid unsupervised chronic steroid use: cataract, pressure elevation and infection are real risks. Severe photophobia or loss of vision should prompt reassessment even when itching and seasonality suggest allergy. A chronic follicular eye infection that repeatedly returns after drops may originate alongside asymptomatic genital infection. Treating the patient and arranging partner care address recurrence. Rapidly reaccumulating purulent discharge is a different, urgent pathway because gonococcus can threaten the cornea.